

Competitor Atlas

Sixteen players in health-, wellbeing- and development-oriented apps and games — one slide each: the problem they solve, their markets and target user, how they ran research, what went wrong, where they are now, and what we can learn. Plus a verdict on Логопотам and the adjacent kids' segment.

Six shelves of the same store

How the 16 players cluster — and where Content Therapy sits.

PRESCRIPTION DTX	EndeavorRx (Akili) · Pear · Big Health — strongest evidence, worst business outcomes; the reimbursement graveyard.
AI CHAT THERAPY	Woebot · Wysa — squeezed between FDA timelines and frontier LLMs.
GOVERNMENT - COMMISSIONED	Kooth — the rare profitable player; sells to states, not app stores.
NARRATIVE & GAMES	eQuoo · Betwixt · SuperBetter · Zombies, Run! — our closest mechanical relatives; all sub-scale.
CONSUMER WELLNESS	Headspace · Calm · Finch · Fabulous · Happify · Lumosity — legally safest, economically brutal; one FTC casualty.
ADJACENT (KIDS DEV/HEALTH)	Логопотам · Сказбука — parent-paid children's development; instructive, not competitive.

POSITIONING TAKEAWAY

Nobody on this map combines adaptive narrative generation + clinical telemetry + device-native screenlife. Our real day-one competitors are not on it at all: ChatGPT + Notes (\$0–20/mo) and doing nothing.

EndeavorRx — Akili Interactive

RX VIDEO GAME · ADHD

SOLD \$34M · -97% vs SPAC

PROBLEM SOLVED Pediatric ADHD attention deficits treated through an action video game — first FDA-cleared (De Novo, 2020) game-as-treatment, adjunctive to or instead of stimulants.	MARKETS US only at prescription scale; Shionogi partnership for Japan/Taiwan never scaled; 2023 pivot to OTC (EndeavorOTC, adults, cleared Jun 2024).	TARGET USER Children 8–12 with ADHD via prescribing physicians; post-pivot: adults with ADHD buying direct.
HOW THEY RESEARCHED Licensed UCSF Gazzaley-lab tech (NeuroRacer). Pivotal STARS-ADHD RCT, n=348, ~20 US sites (Duke PI), objective TOVA endpoint, published in <i>Lancet Digital Health</i>. [FDA]	WHAT WENT WRONG Payers refused coverage, prescribers unfamiliar, Rx volume tiny. 40% layoffs (2023), prescription model abandoned, more cuts 2024. [STAT]	STAGE NOW Acquired by Virtual Therapeutics for ~\$34M (2024) after a ~\$1B SPAC and >\$230M raised (Neuberger, Polaris, Temasek, PureTech). [MedTech Dive]
WHAT WE LEARN The best evidence in the category (Lancet RCT + FDA first) still died on distribution. Design the revenue model before the regulatory strategy — exactly why our ARG + employer surfaces come first.		

Pear Therapeutics

PDT PLATFORM · SUD / INSOMNIA

BANKRUPT · \$1.6B → \$6M

PROBLEM SOLVED

Substance-use disorder, opioid-use disorder and chronic insomnia treated by prescription software (reSET, reSET-O, Somryst — 9-week CBT-I).

MARKETS

US prescription + payer market exclusively — the payer market never arrived.

TARGET USER

Diagnosed clinical patients reached through prescribers and health systems.

HOW THEY RESEARCHED

Full FDA-grade program: RCTs per product, first-ever FDA clearance for a PDT (reSET, 2017); Somryst 510(k) 2020 on SHUTi trial base. [\[FDA\]](#)

WHAT WENT WRONG

~Half of prescriptions written were never reimbursed; no Medicare benefit category; commercial payers declined; burn built for a market that didn't exist. [\[STAT\]](#)

STAGE NOW

Chapter 11 (Apr 2023); assets — including the FDA clearances — auctioned for ~\$6M total. [\[Fierce\]](#)

WHAT WE LEARN

Regulatory clearance without payment rails carries ~zero enterprise value. The CMS G-codes (2025–26) fix the rail Pear died waiting for — but we still treat payer revenue as upside, never as the plan.

Sleepio & Daylight — Big Health

dCBT-I + ANXIETY

ALIVE · THE TRANSFER BLUEPRINT

PROBLEM SOLVED

Chronic insomnia (Sleepio: automated 6-session CBT-I with a virtual "Prof") and worry/anxiety (Daylight) — no human therapist in the loop.

MARKETS

UK NHS (Scotland: national availability, a world first) → US self-insured employers and PBM formularies (Evernorth, CVS Caremark). FDA 510(k) only in Aug 2024.

TARGET USER

Working adults with insomnia/anxiety, reached through employee benefits — not app stores, not prescriptions.

HOW THEY RESEARCHED

Co-founder Colin Espie (Oxford) led 13+ RCTs from a 2012 placebo-controlled trial to the Lancet Psychiatry OASIS trial; NHS real-world evaluation n=7,000+ (56% success); SCI/ISI/GAD-7 instruments; first-ever NICE guidance for a DTx (2022). [\[NICE\]](#)

WHAT WENT WRONG

Nothing fatal — but monetization lagged evidence by ~a decade; nearly every trial is company-affiliated (disclosed COI); users complain about rigidity/billing. [\[STAT\]](#)

STAGE NOW

Private, ~\$127M raised (SoftBank Vision Fund 2 led Series C, 2022); the category's evidence leader.

WHAT WE LEARN

The proven research → market route: UK academic evidence → NICE/NHS credential → US employer dollars. Also our insomnia-first indication logic comes from their playbook — sleep has the clearest instruments and behavioral mechanism.

Woebot Health

SCRIPTED CBT CHATBOT

CONSUMER APP CLOSED JUN 2025

PROBLEM SOLVED

24/7 CBT-based emotional support via a safety-constrained, hand-scripted chatbot — the most clinically serious player in AI therapy.

MARKETS

US consumer (~1.5M users), then enterprise/health-system licensing after the consumer exit.

TARGET USER

Adults wanting free, immediate, stigma-free support between (or instead of) therapy sessions.

HOW THEY RESEARCHED

Founded by Stanford psychologist Alison Darcy; peer-reviewed RCTs (PHQ-9 endpoints); FDA Breakthrough designation for a postpartum-depression product; dropped its FDA application in 2023.

WHAT WENT WRONG

Squeezed from both sides: FDA path too slow and costly for tech that would be obsolete by approval — while free frontier LLMs reset user expectations and made a scripted bot feel archaic. [\[STAT\]](#)

STAGE NOW

Consumer app shut down June 30, 2025 (~\$114M raised); pivoted to B2B licensing.

WHAT WE LEARN

Never compete on conversation quality — that lane belongs to frontier models. Our differentiation must live where LLMs can't follow: device-native screenlife, physician trust, and clinical telemetry.

Wysa

AI CHATBOT + HUMAN COACH

ALIVE · NICHE-STABLE

PROBLEM SOLVED

Stigma-free anonymous mental-health support: CBT chatbot ("penguin") + self-help library + optional human coach escalation.

MARKETS

The rare India → global path: Bangalore build → UK NHS (talking-therapies waitlist support) → US employers/insurers; FDA Breakthrough designation May 2022.

TARGET USER

Adults avoiding formal therapy (stigma, cost, waitlists); employees via benefits; NHS patients on waiting lists.

HOW THEY RESEARCHED

PHQ-9/GAD-7 in trials and deployments; registered US chronic-pain trial (NCT05533190); real-world JMIR analyses; 36+ publications — heavy on engagement data, lighter on completed large RCTs. [\[CT.gov\]](#)

WHAT WENT WRONG

2018 BBC-reported investigation: mishandled disclosures of child abuse and self-harm with generic prompts — the category's canonical crisis-handling failure. [\[analysis\]](#)

STAGE NOW

~\$30M raised (HealthQuad, British International Investment); steady B2B/NHS niche.

WHAT WE LEARN

The anonymity-beats-stigma hypothesis is portable across countries — and crisis handling is the single reputational point of failure: our hard-coded escalation paths are non-negotiable.

Kooth

ANONYMOUS YOUTH SUPPORT

PROFITABLE LISTED PLC

PROBLEM SOLVED

Anonymous digital community + professional chat counselling for young people — commissioned as public infrastructure, free at point of use.

MARKETS

UK NHS integrated care boards (two decades) → US states: California "Soluna" (~\$188M contract), Pennsylvania.

TARGET USER

Young people ~10–25; the actual buyer is a government population-health budget.

HOW THEY RESEARCHED

Service-evaluation evidence, not RCTs: 20 years of NHS service data; bespoke goal-based and session-level outcome measures (built because PHQ-9 fits anonymous drop-in care poorly); peer-reviewed economic evaluation. [\[ScienceDirect\]](#)

WHAT WENT WRONG

US scrutiny: KFF investigation into a California official flown to London by the company; media reports of slow Soluna uptake → share-price plunge and formal rebuttal; one-state concentration risk. [\[KFF\]](#)

STAGE NOW

AIM-listed (IPO 2020, ~£26M); revenue roughly doubled post-California; the category's rare profitable company.

WHAT WE LEARN

The biggest checks in this market are government population contracts — and outcome instruments can be fitted to the care model (theirs to anonymity; ours to narrative). Scrutiny scales with public money.

eQuoo — PsycApps

NARRATIVE RPG · EMOTIONAL FITNESS

SUB-SCALE · £1.5M SEED

PROBLEM SOLVED

Teaching psychological skills ("emotional fitness") to young adults through a choose-your-own-adventure RPG — therapy techniques disguised as game mechanics.

MARKETS

UK (NHS Apps Library listing, 2019) → US universities and schools (B2B), leveraging the NHS badge as a portable trust signal for the US raise.

TARGET USER

Students and young adults 18–28, bought by university student services rather than the users themselves.

HOW THEY RESEARCHED

Large 5-week RCT in a student population, resilience + depression/anxiety scales, published JMIR Mental Health 2023 (UCL co-authors; founder-psychologist Silja Litvin lead author); notably low attrition vs control apps. [\[JMIR\]](#)

WHAT WENT WRONG

Founder-authored evidence (COI), self-report-only outcomes, 5-week follow-up; content-hungry game on a micro budget; the NHS Apps Library itself was wound down, weakening the credential.

STAGE NOW

Operating, sub-scale; £1.5M seed (Morningside, 2022). [\[UKTN\]](#)

WHAT WE LEARN

A published RCT + national health-system listing is a £1.5M problem — the benchmark for our clinical budget line. And: narrative formats show better attrition than worksheet apps — our core bet, already visible here.

Betwixt — Mind Monsters Games

INTERACTIVE FICTION · CBT/ACT

BOOTSTRAPPED INDIE

PROBLEM SOLVED

Self-exploration and emotion regulation for people who find CBT worksheets sterile — a text adventure with soundscapes users compare to Myst.

MARKETS

Global English-language app stores from a UK/Bulgarian founder team; NHS-adjacent research aimed at converting academic validation into B2B credibility.

TARGET USER

Adults seeking emotional engagement that "doesn't feel like therapy"; wellness-curious gamers.

HOW THEY RESEARCHED

Unusually rigorous battery for an indie: University of Nottingham + Lincolnshire NHS Trust single-case study (n=8, completed 2025) with DERS-SF primary + PHQ-9, GAD-7, SCS-SF, SWEMWBS, WSAS. [\[trial record\]](#)

WHAT WENT WRONG

No funding, n=8 evidence, app-store discoverability wall; the founder's own essay concedes mental-health tech's engagement model is broken. [\[essay\]](#)

STAGE NOW

Bootstrapped, tiny team, no disclosed rounds; "two psychology labs" claim is company marketing, not published studies.

WHAT WE LEARN

Validated instruments cost almost nothing — attach them from the first pilot. But distribution, not evidence, kills indie narrative apps: our ARG surface exists precisely to fund distribution.

SuperBetter

GAMEFUL RESILIENCE

STALLED · \$270K LIFETIME

PROBLEM SOLVED

Resilience and recovery (depression, concussion, anxiety) via a "live gamefully" framework — quests, power-ups, bad guys, allies — born from Jane McGonigal's own concussion recovery.

MARKETS

US → global consumer (English); B2B2C pivot to schools and health systems (Parkland Health, Dallas); a Zambia children's pilot.

TARGET USER

Teens and adults recovering from setbacks; students via school districts.

HOW THEY RESEARCHED

Third-party academic trials: University of Pennsylvania RCT (n=283, CES-D/GAD-7, 2015) and an NIH-funded Ohio State pediatric concussion pilot — genuine independence, rare in this category. [\[PubMed\]](#)

WHAT WENT WRONG

The venture-backed company wound down mid-2010s; the product survives as a lean LLC; flagship evidence is one small, dated, high-dropout study; ~\$270K lifetime funding. [\[Republic\]](#)

STAGE NOW

Lean LLC, equity-crowdfunded 2022; pursuing schools/health-system deals.

WHAT WE LEARN

A bestselling book + one independent RCT ≠ a company. Evidence has a shelf life — plan continuous, funded research, not a single validation moment.

Zombies, Run! — Six to Start

NARRATIVE AUDIO FITNESS · ~10M USERS

NEAR-DEATH, RESCUED 2025

PROBLEM SOLVED

Making running motivating: a zombie-apocalypse audio drama that turns every run into a story mission — the biggest smartphone fitness game ever.

MARKETS

UK Kickstarter (2012) → global app stores from day one; story content is language-bound (English), capping localization.

TARGET USER

Runners and fitness-avoiders who respond to narrative motivation rather than metrics.

HOW THEY RESEARCHED

None company-run — a hit consumer product first; UCL researchers later studied user engagement qualitatively (Games for Health Journal, co-creator as co-author). [\[PubMed\]](#)

WHAT WENT WRONG

Content-treadmill economics (seasons are expensive, subscription niche); studio sold for \$9.5M to OliveX/Animoca (2021); crypto-direction owner cut the team to two, stalled the story mid-cliffhanger. [\[Techbuzz\]](#)

STAGE NOW

Co-creator Naomi Alderman bought it back (Nov 2025); independent again.

WHAT WE LEARN

Narrative drives real-world behavior at 10M-user scale — proof our core mechanism works. But hand-authored story content is a treadmill: our parameterized template engine exists to break exactly this cost curve.

Lumosity — Lumos Labs

BRAIN TRAINING · 85M USERS AT PEAK

FTC \$2M · 2016

PROBLEM SOLVED (CLAIMED)

"Train your brain": memory/attention mini-games marketed as improving work, school and athletic performance and delaying cognitive decline.

MARKETS

US → global consumer subscription; ~\$70M raised (Discovery Communications, Menlo Ventures).

TARGET USER

Adults anxious about cognitive decline — the FTC said the marketing "preyed on consumers' fears."

HOW THEY RESEARCHED

Proprietary in-game indices (Lumosity Performance Index) + a data-sharing "Human Cognition Project" — no independent RCT validation of the claims. In 2014, ~70 scientists convened by Stanford Longevity Center + Max Planck issued a consensus against brain-game transfer claims. [\[SciAm\]](#)

WHAT WENT WRONG

FTC deceptive-advertising action: \$50M judgment, \$2M paid (Jan 2016); order requires RCT-grade "competent and reliable scientific evidence" for any health/performance claim — the category's wording rulebook ever since. [\[FTC\]](#)

STAGE NOW

Still operating, diminished; growth never recovered; "you get better at the game, not at life" became the standard critique.

WHAT WE LEARN

The Lumosity order is effectively our compliance spec: engagement language is free, outcome language requires trial data, testimonials must be clean, cancellation must be easy.

Happify / Twill

GAMIFIED POSITIVE PSYCHOLOGY

SOLD \$22.4M AFTER \$118M RAISED

PROBLEM SOLVED

Positive-psychology + CBT activities packaged as game "tracks"; later rebranded Twill as an enterprise "Intelligent Healing" platform for chronic conditions.

MARKETS

US consumer → US enterprise (health plans, pharma co-promotions); multiple app-store languages; no regulatory pathway completed anywhere.

TARGET USER

Employees and health-plan members with subclinical stress/low mood; pharma partners' patient populations.

HOW THEY RESEARCHED

In-house science team publishing real-world observational analyses (subjective wellbeing score + symptom scales), e.g. a peer-reviewed study in adults with cardiovascular risk — self-selected users, acknowledged limitation. [\[PMC\]](#)

WHAT WENT WRONG

Stuck in the middle: neither consumer love (vs Calm) nor clearance (vs Pear); enterprise sales couldn't sustain the burn; distress-era consolidation. [\[STAT\]](#)

STAGE NOW

Acquired by DarioHealth for ~\$22.4M in stock+notes (Feb 2024) — >80% value destruction.

WHAT WE LEARN

The "science-ish" middle position is the worst seat at the table. Each of our surfaces commits fully: ARG = pure entertainment, consumer = pure wellness, clinical = real trials.

Headspace

MEDITATION → CARE PLATFORM B2B RETREAT · 3 LAYOFF ROUNDS

PROBLEM SOLVED Making meditation accessible and habitual (ex-monk Andy Puddicombe's guided content); post-Ginger merger: full-stack coaching/teletherapy/EAP.	MARKETS UK-founded, scaled from LA to 70+ countries via app stores — a marketing-led transfer with no regulatory anchor; then US employer/health-plan channel.	TARGET USER Mainstream consumers; post-2021, HR buyers at large employers.
HOW THEY RESEARCHED Research followed product: partner studies (UCSF digital-meditation RCT for employee stress; PSS/burnout self-report) + an open call for external researchers — issued because cognitive-benefit evidence was inconclusive. [Fierce]	WHAT WENT WRONG \$3B Ginger merger indigestion + post-COVID demand normalization: ~50 cut Dec 2022, 181 (15%) Jul 2023 incl. 33 therapists overnight, 13% more Nov 2024; therapist network moved to contract roles. [BHB]	STAGE NOW Private (~\$215M + Ginger ~\$220M raised); B2B-first; regulatorily clean throughout.
WHAT WE LEARN Brand + content library isn't defensibility — the employer channel is where revenue stabilized. Plan B2B distribution from month 6, not as a plan-B.		

Calm

SLEEP & RELAXATION · CATEGORY LEADER

POST-COVID SLUMP

PROBLEM SOLVED

Sleep and stress relief through content: Sleep Stories (celebrity-narrated), meditations, soundscapes — entertainment mechanics applied to wellness before anyone else.

MARKETS

US → global consumer app stores; Calm Business / Calm Health for employers and health plans.

TARGET USER

Consumers with sleep trouble and everyday stress — the broadest audience on this map.

HOW THEY RESEARCHED

Self-funded academic trials at Arizona State (PSS, sleep-disturbance measures, JMIR/PLOS One RCTs) — whose lead researcher then became Calm's head of science; product itself tracks engagement, not clinical scores. [\[JMIR\]](#)

WHAT WENT WRONG

Post-COVID engagement contraction: 20% layoffs (Aug 2022, "grew too fast"); Head of Science departed weeks later — the clinical-evidence push was deprioritized. [\[STAT\]](#)

STAGE NOW

Private; ~\$218M raised at \$2B peak (Lightspeed, Goldman, TPG); profitable early, retrenched since.

WHAT WE LEARN

Sleep is the wedge with the clearest consumer willingness-to-pay — supporting our insomnia-first indication. And even the #1 brand hit the churn ceiling: engagement mechanics beat content libraries.

Finch

SELF-CARE PET • GEN-Z HIT

GROWING ORGANICALLY

PROBLEM SOLVED

Shame-free self-care habit formation: a virtual pet bird that grows when you check in, journal, breathe, or do micro-goals — care for yourself by caring for it.

MARKETS

US-built → global, spread almost entirely by organic TikTok/word-of-mouth — near-zero paid CAC.

TARGET USER

Gen-Z with anxiety/ADHD/depression who abandoned streak-punishing productivity apps — "the first self-care app they've kept using."

HOW THEY RESEARCHED

They didn't — no published studies, no clinical instruments; design borrows behavioral-activation/CBT-adjacent techniques informally. [\[review\]](#)

WHAT WENT WRONG

Nothing yet — but zero clinical evidence caps any health-budget access; monetization deliberately limited to cosmetics (~\$9.99/mo Finch Plus); dense UI for newcomers.

STAGE NOW

Growing indie Public Benefit Corporation; funding not public.

WHAT WE LEARN

Live proof of our gamification finding: celebration-not-punishment mechanics retain the exact clinical population we target. Finch validates the mechanic but can't monetize health budgets — we attach the instruments they skipped.

Fabulous

HABIT COACH · BEHAVIORAL ECONOMICS

BOOTSTRAP-PROFITABLE, EVIDENCE-FREE

PROBLEM SOLVED

Building morning routines and healthy habits through structured "journeys" and rituals, grounded in behavioral-economics design (tiny habits, loss aversion).

MARKETS

Global consumer app stores (team roots Paris/Tunisia, US-facing); no regulatory pathway anywhere.

TARGET USER

Self-development consumers; Apple editorial darling (Design Award finalist positioning).

HOW THEY RESEARCHED

Incubated at Duke's Center for Advanced Hindsight (Dan Ariely) — but transferred *design principles*, not evidence: its own science page names zero peer-reviewed studies of the app. [\[Fabulous\]](#)

WHAT WENT WRONG

"Science-based" branding without studies drew methodical criticism; aggressive onboarding paywall and cancellation friction are recurring complaints. [\[critique\]](#)

STAGE NOW

Largely bootstrapped/revenue-funded; no large disclosed rounds.

WHAT WE LEARN

Lab pedigree ≠ evidence — the exact credibility trap our pre-mortem R10 guards against. Prestigious origins buy one meeting; only instruments and trials buy the second.

Логопотам (Logopotam)

ONLINE SPEECH THERAPY FOR KIDS · RU

VERDICT: ADJACENT, NOT A COMPETITOR

PROBLEM SOLVED

Children's speech disorders (~30–40% of preschoolers): live online lessons with certified speech therapists + a gamified exercise app (coins, stickers, missions) + an AI diagnostic "virtual therapist" in development. [\[vc.ru\]](#)

MARKETS & USER

Russia / Russian-speaking families; parents of kids ~3–8 buying lesson packages. Scale (self-reported 2024–25): ~23K app users, ~2.7K school students, 500+ therapists, ~20%/mo growth. [\[T-Bank\]](#)

STAGE & MONEY

~40M RUB (~\$450K) round — Datalab, АиК-Инвест, private investors; founded 2020 by Alexey Litvinov; early-growth, seed-scale.

HOW THEY VALIDATE

Certified logopedic methodology + therapist assessment as ground truth; AI diagnostics benchmarked against human evaluation (85–95% accuracy claim); before/after videos and parent dashboards — service-grade validation, no published trials.

THEIR PROBLEMS

Service-heavy economics (humans in every lesson), rigid scheduling complaints, some no-result complaints after months, pushy trial follow-ups, online-attention limits with small kids. [\[reviews\]](#)

WHY NOT OUR CATEGORY

Different clinical domain (speech-skill correction vs emotional/mental health), different mechanism (drill exercises + live human lessons vs adaptive narrative content), different buyer dynamics (parent-purchased pediatric service). Overlap is only "gamified, parent-paid kids' health-and-development." Same shelf-level: Сказбука (Zebrainy → Yandex) for early learning.

WHAT WE STILL LEARN FROM IT

Parent willingness-to-pay for children's health-development is real and recurring; and the "human specialist + gamified app homework" hybrid is a working template for our clinician-supervised surface. Watch, borrow the hybrid model — don't count it as competition.

What the whole map teaches us

- 01 The payer is the product-market fit. Best evidence (Akili, Pear) died; consumer/employer/government sellers survived. Our sequence — ARG → consumer → employers → clearance — follows the survivors.
- 02 The proven transfer route is UK evidence → NICE/NHS credential → US enterprise dollars (Big Health, Kooth, eQuoo). Wysa shows India → global works with an English core. Nobody springboarded off Germany's DiGA.
- 03 Research design splits the map: DTx players used validated instruments (TOVA, ISI, PHQ-9) and RCTs; consumer winners measured streaks. The middle path that works at seed cost: games with real instruments attached (eQuoo £1.5M, Betwixt's battery).
- 04 Almost all evidence is company-affiliated (Espie/Sleepio, Huberty/Calm, Litvin/eQuoo). Independent PIs are the cheapest differentiation available to us.
- 05 Claims discipline is survival: Lumosity wrote the FTC rulebook; Fabulous shows pedigree-without-studies erodes trust; Finch shows claiming nothing is safe but locks out health budgets.
- 06 Narrative mechanics are validated — by Zombies, Run! at 10M users, by Finch's retention with our exact clinical population, by eQuoo's low attrition. What no one has: adaptivity + clinical telemetry. That's our slot.
- 07 Content cost is the silent killer of story products (Zombies' treadmill). The parameterized template engine is not a nice-to-have; it's the unit-economics thesis.
- 08 Exit reality: disclosed exits cluster at \$6–34M against \$2–3B paper peaks. The credible Series A story is seed-scale evidence + contracted B2B revenue — which is precisely the deck we built.